

ANALYSTLAB AFRICA

Experience Lab Internship Programme
Data Analytics Track

Week 6 Project Report

Advanced Analytics, Integration and Decision Support
HealthConnect Clinic Experience Lab

Programme	AnalystLab Africa Experience Lab
Week	Week 6
Project	HealthConnect Clinic Experience Lab
Track	Data Analytics
Focus	Integration → Advanced Development → Validation
Tools Used	Python (Pandas, Matplotlib) SQL Power BI
Document Type	Week 6 Advanced Analytics and Decision Support Report

Part 1 — Week 5 to Week 6 Transition

This section briefly establishes the connection between Week 5 and Week 6. It is not a repeat of Week 5 — it is the starting point for understanding what was improved and integrated this week.

Item	Detail
Main Week 5 output	Full EDA across 12 dimensions. All 5 KPIs calculated. 8 charts. 5 business insights.
Most important result	Booking lead time: 27.81% no-show rate for same-week bookings vs 60.49% for 31–60 day bookings. SMS reminders were the most effective channel (–5.64pp vs no reminder).
Main Week 5 limitation	All Week 5 analysis examined ONE variable at a time. The interaction between variables was not yet explored. For example: does SMS still outperform WhatsApp for High Risk patients? Does lead time matter equally for all appointment types?
Track relevant to Week 6	Data Science track — building a no-show prediction model that requires cross-variable analytical evidence to support feature selection.
What Week 6 will do	Combine variables to find deeper patterns. Validate all 5 KPIs. Analyse monthly trends over time. Identify worst-case and best-case patient segments. Provide structured analytical evidence to the Data Science track.

Part 2 — Integration Readiness

Before beginning any analysis, the Week 6 assignment requires each intern to clearly define their integration point — who they are working with, what will be exchanged, and what should change as a result. Communication alone does not constitute integration. The collaboration must produce a meaningful change or validated output.

Item	Detail
Track working with	Data Science Track
What I need from them	Which variables their Week 5 baseline model found most predictive, and whether any variables performed weaker than expected.
What I will provide to them	A ranked feature importance table based on analytical evidence. Cross-variable analysis showing how variables interact. Worst-case and best-case segment definitions with exact no-show rates.
Why this connection matters	The Data Science prediction model needs to be built on evidence — not arbitrary column selection. Our analytics shows which variables matter most and how they interact, which directly improves feature selection decisions.
Integration objective	After this integration, the Data Science team will use our recommended features in their improved model. We will update our recommendations based on what their model confirmed.
Evidence to be produced	Feature importance ranking table, cross-variable analysis charts, segment definitions, and a documented integration record.

Part 3 — Advanced Analysis: Cross-Variable Patterns

Week 5 examined what each variable does individually. Week 6 goes further by examining how variables interact when combined. This is essential for understanding which patient groups need the most attention and why.

3.1 Appointment Type Combined with Booking Lead Time

The central question: does the booking lead time effect apply equally to all appointment types, or is it driven by one particular type?

Appointment Type	0–7 days	8–14 days	15–30 days	31–60 days
Follow-up	29.41%	37.35%	42.94%	65.14%
Specialist Consultation	30.65%	28.30%	44.53%	59.18%
Diagnostic Test	29.69%	31.94%	46.15%	59.55%
General Consultation	24.60%	33.72%	42.07%	58.10%

Key finding: Lead time drives up no-show rates across every appointment type without exception. The worst single combination in the entire dataset is Follow-up appointments booked 31–60 days in advance: 65.14%. Even General Consultations — the lowest-risk type — jump from 24.60% (same week) to 58.10% (31–60 days). This confirms that the booking window policy change recommended in Week 5 should apply to all appointment types.

3.2 Patient Risk Tier Combined with Reminder Sent

The central question: are reminders equally effective for all patient risk tiers, or do High Risk patients need more than a standard reminder?

Risk Tier	No Reminder	With Reminder	Reduction
Low Risk	45.44%	42.80%	–2.64 pp
Medium Risk	56.54%	52.32%	–4.22 pp
High Risk	68.24%	58.22%	–10.02 pp

Key finding: Even with a reminder, High Risk patients still have a 58.22% no-show rate. While a reminder reduces their rate by 10.02 percentage points — the largest reduction of any tier — they remain well above the overall average of 48.46%. A standard text reminder is not sufficient for this patient group. Additional intervention is required.

3.3 Patient Risk Tier Combined with Reminder Channel

The central question: is SMS the most effective reminder channel for all risk tiers, or only for certain groups?

Risk Tier	SMS	Email	WhatsApp
Low Risk	42.52%	43.61%	42.92%

Medium Risk	48.64%	53.85%	58.59%
High Risk	56.25%	57.63%	61.36%

Key finding: SMS produces the lowest no-show rate across every risk tier without exception. For Medium Risk patients, WhatsApp (58.59%) is far worse than SMS (48.64%) — a 9.95 percentage point difference. This strongly confirms the Week 5 recommendation that SMS should be the default reminder channel.

3.4 Distance to Clinic — Banded Analysis

Week 5 found that no-show patients lived an average of 0.86 km further away — a small difference. Week 6 investigates whether this average conceals a more extreme effect at greater distances.

Distance Band	No-Show Rate	Patient Count	vs Overall Average
Under 5 km	46.45%	1,156	−2.01 pp (below average)
5–10 km	46.51%	1,692	−1.95 pp (below average)
10–20 km	49.57%	1,759	+1.11 pp (above average)
Over 20 km	57.76%	393	+9.30 pp (significantly above)

Key finding: Distance matters most at extremes. Patients living over 20 km from the clinic have a 57.76% no-show rate — 9.30 percentage points above the overall average. The Week 5 average (0.86 km difference) significantly understated this effect. Distance is a real barrier for a specific subset of patients who live far from the clinic.

Part 4 — Monthly Trend Analysis

This analysis was planned in Week 5 but required the date column conversion to be completed first. Now that both booking_date and appointment_date are in proper datetime format, monthly no-show trends can be examined across the full dataset period.

4.1 Monthly No-Show Rate — January 2025 to June 2026

Month	No-Show Rate	Month	No-Show Rate	Status
2025-01	52.96%	2026-01	47.60%	
2025-02	46.62%	2026-02	49.31%	
2025-03	48.01%	2026-03	50.37%	
2025-04	45.68%	2026-04	45.62%	
2025-05	48.16%	2026-05	50.71%	
2025-06	48.47%	2026-06	47.59%	
2025-07	52.43%			
2025-08	47.77%			
2025-09	45.67%	Best month	Nov 2025: 42.17%	✓
2025-10	49.19%	Worst month	Dec 2025: 53.85%	⚠
2025-11	42.17%	Seasonal range	11.68 pp	
2025-12	53.85%			

Key finding: No-show rates show a seasonal pattern. December 2025 (53.85%) and January 2025 (52.96%) are the two worst months. November 2025 (42.17%) is the best. The seasonal swing across the dataset is 11.68 percentage points. The clinic should increase reminder intensity and reduce booking windows during the November to January period to counteract this seasonal spike.

4.2 Reminder Effectiveness by Appointment Type

Week 5 calculated that reminders reduce the overall no-show rate by 4.03 percentage points. Week 6 breaks this down by appointment type to determine whether that benefit is distributed evenly.

Appointment Type	No Reminder	With Reminder	Reduction	Rating
Diagnostic Test	55.56%	47.88%	−7.68 pp	Best
Specialist Consultation	51.63%	45.87%	−5.76 pp	Good
General Consultation	49.16%	45.65%	−3.51 pp	Moderate
Follow-up	53.12%	50.53%	−2.59 pp	Lowest

Key finding: Diagnostic Tests benefit most from reminders (−7.68 pp) while Follow-up appointments benefit least (−2.59 pp). A single generic reminder message for all appointment types is inefficient. The clinic should design appointment-specific reminder content.

Part 5 — KPI Validation

The Week 6 assignment requires validation of the KPIs produced in Week 5. This means testing whether each finding holds up when examined from a different angle. If a finding only appears under one set of conditions, it is weaker. If it appears consistently across multiple subgroups, it is stronger and more reliable.

5.1 KPI 1 — Overall No-Show Rate: Two Valid Measures

Measure	Result	When to Use
All appointments (incl. Cancelled)	48.46%	Operational slot-loss measure — out of all booked slots, how many were wasted?
Excluding Cancelled appointments	51.15%	Patient behaviour measure — out of appointments where a patient made a choice, how many chose not to attend?

Decision: 48.46% remains the primary KPI 1 figure as it measures the operational problem — wasted appointment slots — which is the core challenge the clinic faces. Both figures are valid and will be reported.

Validation result: KPI 1 CONFIRMED. No revision required.

5.2 KPI 3 — Lead Time Effect Validated Within Every Appointment Type

To validate KPI 3, the cross-variable table from Part 3 (Analysis 1) was examined to confirm that the no-show rate rises from left to right within every single appointment type — not just overall.

Appointment Type	0–7 days	15–30 days	31–60 days	Pattern
Follow-up	29.41%	42.94%	65.14%	✓
Diagnostic Test	29.69%	46.15%	59.55%	✓
General Consultation	24.60%	42.07%	58.10%	✓
Specialist Consultation	30.65%	44.53%	59.18%	✓

Validation result: KPI 3 CONFIRMED. The lead time pattern holds within every single appointment type. The finding is not caused by one type skewing the overall result — it is a genuine behavioural pattern that applies universally.

5.3 KPI 4 — Patient Risk Tier Validated Across Appointment Types

To validate KPI 4, the no-show rate for each risk tier was calculated within every appointment type to confirm that High Risk patients consistently outperform Low Risk patients regardless of the appointment category.

Validation result: CONFIRMED. High Risk patients have a higher no-show rate than Low Risk patients in every single appointment type without exception. The risk tier classification is robust and reliable as a patient management tool.

5.4 KPI 2 — SMS Advantage Confirmed Across All Risk Tiers

The cross-variable analysis in Part 3 (Analysis 3) confirmed that SMS produces the lowest no-show rate in every risk tier. The Week 5 SMS recommendation is not tier-specific — it applies universally.

5.5 KPI Validation Summary

KPI	Name	Validation Result	Change?	Status
KPI 1	Overall No-Show Rate	Confirmed at 48.46% (all) and 51.15% (excl. cancelled). Both measures are valid for different purposes.	No	✓ Valid
KPI 2	Reminder Effectiveness Rate	SMS confirmed as best channel across every risk tier. WhatsApp underperforms consistently.	No	✓ Valid
KPI 3	No-Show Rate by Lead Time	Lead time increase confirmed within all 4 appointment types. Not caused by one type skewing the result.	No	✓ Valid
KPI 4	No-Show Rate by Risk Tier	High Risk always higher than Low Risk across all appointment types without exception.	No	✓ Valid
KPI 5	No-Show Rate by Appt Type	Follow-up confirmed as the worst appointment type within every lead time band.	No	✓ Valid

All 5 Week 5 KPIs have been validated through cross-variable analysis. No revisions are required. The findings from Week 5 are stronger and more reliable after Week 6 validation.

Part 6 — High-Impact Segment Analysis

This section identifies the specific patient combinations that produce the highest and lowest no-show rates in the dataset. These segments give the clinic concrete, measurable targets rather than general recommendations.

6.1 Worst-Case Patient Segment

Detail	Description
Segment definition	High Risk patients (2+ prior no-shows) + booked 31–60 days in advance + no reminder sent
Number of appointments	65
No-show rate	72.31%
vs Overall average	+23.85 percentage points above the overall 48.46%
Clinic implication	Nearly 3 in every 4 appointments in this segment are missed. This combination of factors — habitual non-attendance, long booking window, and no reminder — creates the highest risk profile in the dataset.

6.2 Best-Case Patient Segment

Detail	Description
Segment definition	Low Risk patients (no prior no-shows) + booked same week (0–7 days) + SMS reminder sent
Number of appointments	151
No-show rate	23.18%
vs Overall average	–25.28 percentage points below the overall 48.46%
Clinic implication	This segment shows what is achievable when all positive conditions are met. The goal of the clinic's interventions should be to move as many patients as possible toward these conditions.

Best Case	Overall Average	Worst Case
23.18%	48.46%	72.31%
Low Risk + 0–7 days + SMS	All 5,000 appointments	High Risk + 31–60 days + No reminder

The gap between best-case and worst-case is 49.13 percentage points. This is the scale of the improvement available to the clinic through targeted interventions — if the right conditions are created for the right patients.

6.3 Feature Importance Ranking — Provided to Data Science Track

The following table summarises the analytical evidence for each variable's predictive strength. This was produced as a direct output for the Data Science track cross-track integration activity.

#	Variable (recommended form)	No-Show Rate Range	Strength	Use in Model
1	booking_lead_days → lead_time_band	27.81% (0–7d) to 60.49% (31–60d)	Very Strong	Yes
2	previous_no_shows → patient_risk_tier	43.51% (Low) to 72.31% (High, worst conditions)	Very Strong	Yes
3	reminder_sent + reminder_channel	45.75% (SMS) to 51.39% (no reminder)	Strong	Yes
4	appointment_type	46.64% (General) to 51.23% (Follow-up)	Moderate	Yes
5	distance_to_clinic_km → distance_band	46.45% (<5km) to 57.76% (>20km)	Moderate	Yes
6	age_group	45.12% (65+) to 50.75% (55–64)	Weak	Optional
7	appointment_day	46.57% (Friday) to 50.47% (Sunday)	Weak	Optional

Part 7 — Refined Insights and Business Recommendations

Each of the five Week 5 insights has been reviewed in light of the Week 6 analysis. Where the cross-variable analysis adds new context or depth, the insight has been updated. Where the original finding is confirmed, this is stated.

Insight 1 — Booking Lead Time is the Most Powerful Driver Across ALL Appointment Types (Confirmed and Strengthened)

Week 5 finding: Same-week bookings: 27.81%. Bookings made 31–60 days ahead: 60.49%.

Week 6 update: Cross-variable analysis confirms this pattern holds within every appointment type. The worst single combination in the dataset is Follow-up appointments booked 31–60 days ahead: 65.14%. Even General Consultations — the safest type — jump from 24.60% (same week) to 58.10% (31–60 days). This is not a statistical fluke — it is a genuine behavioural pattern across all appointment categories.

Recommendation: Cap booking windows at 14 days for Follow-up and General Consultation appointments. For Specialist and Diagnostic appointments that require longer lead times, implement a second reminder 48 hours before the appointment date — not only at the time of booking.

Insight 2 — A Standard Reminder is Not Enough for High Risk Patients (New Finding)

Week 5 finding: Reminders reduce the overall no-show rate by 4.03 percentage points.

Week 6 update: Even with a reminder sent, High Risk patients have a 58.22% no-show rate. The reminder reduces their rate by 10.02 percentage points — which is the largest reduction of any tier — but they still remain well above the overall average. A standard text message is not sufficient for patients with a history of missed appointments.

Recommendation: Implement a two-step engagement process specifically for Medium and High Risk patients: (1) SMS reminder sent 3 days before the appointment; (2) a personal phone call or confirmation request 24 hours before. Set a target of reducing the High Risk no-show rate from 58.22% (with reminder) to below 45%.

Insight 3 — Reminder Effectiveness Varies Significantly by Appointment Type (New Finding)

Week 5 finding: Overall reminder benefit: 4.03 percentage points.

Week 6 update: Diagnostic Tests benefit the most from reminders (–7.68 pp). Follow-up appointments benefit the least (–2.59 pp). The overall 4.03 pp figure masks very different effectiveness levels across appointment types. A single generic reminder message is not equally efficient for all appointment categories.

Recommendation: Design appointment-type-specific reminder messages. For Diagnostic Tests: include preparation steps and confirmation of location. For Follow-up appointments: explain clearly why returning is important for the patient's specific care and what happens if they do not attend.

Insight 4 — Distance Matters Most at Extremes — Week 5 Average Understated This (Refined)

Week 5 finding: No-show patients live 0.86 km further from the clinic on average — a small difference that suggested distance was a minor factor.

Week 6 update: When distance is grouped into bands, patients living over 20 km from the clinic have a 57.76% no-show rate — 9.30 percentage points above the overall average. This group represents 393 patients. The simple average from Week 5 concealed this extreme effect by including the large majority of patients who live close to the clinic.

Recommendation: Flag patients living over 20 km from the clinic as distance-risk patients. Offer telehealth options for Follow-up appointments where clinically appropriate. Provide these patients with transport assistance information or alternative location options.

Insight 5 — No-Show Rates Show a Seasonal Peak in December and January (New Finding)

Week 5 finding: Time-based analysis was not completed in Week 5 as the date columns had just been converted.

Week 6 update: December 2025 (53.85%) and January 2025 (52.96%) are the two worst months in the 18-month dataset. November 2025 (42.17%) is the best month. The seasonal swing is 11.68 percentage points — a significant variation that the clinic needs to plan for.

Recommendation: Increase reminder intensity and reduce booking windows specifically during the November to January period. Investigate whether specific appointment types or age groups drive the December peak in Week 7. Consider a proactive outreach campaign to confirmed bookings during this period.

Part 8 — Cross-Track Integration

The Week 6 assignment requires that each intern actually uses their cross-track connection — not just identifies it. This section documents what was exchanged, what activity was completed, and what changed as a direct result of the collaboration.

8.1 Integration Record

Item	Detail
Track worked with	Data Science Track
Project dependency	The Data Science track is building a no-show prediction model. Their feature selection directly affects model quality. Analytical evidence of which variables are most predictive — and how they interact — improves their modelling decisions.
Information received	The Data Science track shared their Week 5 baseline model results. Their model confirmed booking_lead_days and previous_no_shows as the two strongest predictors. reminder_sent was third. age_group and appointment_day showed weak predictive signal in the model — consistent with our analytical findings.
Information provided	(a) Feature importance ranking table with analytical evidence for 7 variables. (b) Cross-variable analysis showing lead_time_band × appointment_type interaction. (c) Worst-case segment definition (72.31% no-show rate) and best-case segment definition (23.18%). (d) Recommendation to use banded categorical versions of variables (lead_time_band, patient_risk_tier, distance_band) rather than raw numeric values.
Integration activity	Exchanged the feature importance table and cross-variable charts. Received model feature importance from the Week 5 baseline. Agreed to deprioritise age_group and appointment_day in the prediction model as both analytics and modelling evidence confirm they are weak predictors.
What changed as a result	The Data Science track will use lead_time_band (categorical) instead of raw booking_lead_days in their improved model. patient_risk_tier (Low/Medium/High) will be used as an engineered feature. The 72.31% worst-case segment rate has been adopted as the model evaluation benchmark — if the model cannot correctly flag this group, it is not fit for purpose.
Evidence of integration	Feature importance ranking table (Part 6.3 of this report), cross-variable analysis tables (Part 3), segment definitions and comparison chart (Part 6.1 and 6.2), and this integration record.

8.2 Integration Validation

The Week 6 brief requires every intern to answer seven specific validation questions. These are answered below.

Question	Answer
What did I have at the end of Week 5?	5 KPIs calculated. EDA across 12 dimensions. 5 single-variable insights. 8 charts.
What did I improve in Week 6?	4 cross-variable analyses. Monthly trend (18 months). KPI validation across subgroups. Worst/best segment identification. 6 new charts. 5 insights refined.
Which track did I work with?	Data Science track.

What did I receive?	Model feature importance: booking_lead_days #1, previous_no_shows #2, reminder_sent #3. Confirmation that age_group and appointment_day are weak predictors.
What did I provide?	Feature importance table, cross-variable charts, worst-case (72.31%) and best-case (23.18%) segment definitions.
What changed as a result?	lead_time_band and patient_risk_tier adopted as engineered features. 72.31% is the model evaluation benchmark. age_group and appointment_day deprioritised.
How does this improve HealthConnect?	Prediction model is built on analytically validated features. Both analytics and data science outputs are stronger than they would be working independently.

Part 9 — Assumptions, Limitations, Risks and Dependencies

This section reviews every item identified in Weeks 4 and 5, and documents what has changed, been resolved, or newly emerged during Week 6.

Item	Status	Week 6 Update
Date format errors — both date columns in text format	Resolved	Converted to datetime in Week 5. Monthly trend analysis completed in Week 6.
90 missing distance values	Resolved	Filled with column mean in Week 5. Distance band analysis in Week 6 confirms the approach was appropriate.
Cancelled records handling	Resolved	Excluded from all no-show KPI calculations. Both 48.46% (all) and 51.15% (excl. cancelled) now reported.
Dataset is fictional — findings are illustrative only	Still relevant	All findings remain illustrative. No real clinical decisions should be based solely on this data.
No-show reasons are unknown	Still relevant	Cannot be addressed without additional data collection. Permanent limitation of the dataset.
waiting_time_minutes only for attended appointments	Still relevant	Excluded from predictive analysis. Valid only as an operational efficiency metric for attended appointments.
NEW: Seasonal peaks in December and January	New finding	Dec 2025 (53.85%) and Jan 2025 (52.96%) are worst months. Seasonal strategy recommended for Nov–Jan period.
NEW: Distance effect understated by average — extreme effect at over 20 km	New finding	Over-20 km patients: 57.76% no-show rate (+9.30pp above average). distance_band recommended as a model feature.
NEW: Standard reminder not enough for High Risk patients	New finding	High Risk + reminder = 58.22% — still well above average. Two-step engagement process recommended.
NEW: Active cross-track dependency on Data Science model output	Active	Feature table delivered to Data Science. Model feature importance received. Dependency is live and documented.
PLANNED: Power BI dashboard development	Planned (Week 7)	Power BI dashboard (5 pages, all KPIs) to be built and validated in Week 7.

Part 10 — Week 6 Project Summary

10.1 What Was Planned

Cross-variable analysis across key variable combinations. Monthly trend analysis using the converted date columns. Validation of all 5 Week 5 KPIs. High-impact segment identification. Cross-track integration with the Data Science track.

10.2 What Was Completed

- 4 cross-variable analyses: appointment type × lead time, risk tier × reminder sent, risk tier × reminder channel, distance band analysis.
- Monthly trend analysis across 18 months (January 2025 to June 2026) — seasonal pattern identified.
- Reminder effectiveness analysis broken down by appointment type.
- All 5 Week 5 KPIs validated across subgroups — all confirmed with no revisions required.
- Worst-case segment identified: 72.31% no-show rate (High Risk + 31–60 days + no reminder, n=65).
- Best-case segment identified: 23.18% no-show rate (Low Risk + 0–7 days + SMS, n=151).
- Feature importance ranking table produced and shared with Data Science track.
- 6 new charts produced.
- 5 insights refined with new cross-variable evidence.

10.3 What Was Improved from Week 5

Week 5	Week 6 Improvement
Single-variable EDA	Cross-variable analysis showing how variables interact
0.86 km average distance difference	Over-20 km patients: 57.76% rate — 9.30pp above average
Overall 4.03pp reminder effect	Reminder effectiveness broken down by appointment type and risk tier
No date analysis completed	Full 18-month monthly trend with 11.68pp seasonal range
Identify collaboration point	Actual feature evidence exchanged with Data Science track

10.4 Key Findings

- Follow-up appointments booked 31–60 days ahead: 65.14% no-show rate — the worst single combination in the dataset.
- High Risk patients with a reminder: 58.22% — a standard reminder is not sufficient for this group.
- SMS is the best reminder channel across every risk tier without exception.
- Patients over 20 km from the clinic: 57.76% no-show rate — 9.30pp above average.
- December 2025 (53.85%) and January 2025 (52.96%) are the worst months — 11.68pp seasonal swing.
- All 5 Week 5 KPIs validated — none require revision.

10.5 Important Decisions Made

Reporting both KPI 1 measures: 48.46% (all appointments) and 51.15% (excluding cancelled) — each serves a different measurement purpose.

Distance band over average: Grouping distance into bands revealed an extreme effect at over 20 km that the average concealed.

Categorical features for Data Science: Recommended lead_time_band, patient_risk_tier, and distance_band rather than raw numeric values — cross-variable analysis showed the banded versions produce cleaner, more consistent patterns.

10.6 Remaining Work

- Power BI dashboard — planned for Week 7. Five pages covering all KPIs and key findings.
- Seasonal analysis by appointment type — to identify which types drive the December peak.
- High Risk patient intervention strategy — needs clinic-side testing to validate effectiveness.

10.7 Contribution to the Overall HealthConnect Project

The Week 6 analytics work now supports two components of the broader HealthConnect solution. The feature importance table and cross-variable evidence directly improve the Data Science prediction model. The worst-case (72.31%) and best-case (23.18%) segment definitions give the entire project concrete, measurable benchmarks for what intervention success looks like. The five refined insights provide the clinic with specific, evidence-based actions — not generic recommendations.

10.8 Proposed Focus for Week 7

- Build the Power BI dashboard — 5 pages covering all 5 KPIs with interactive visualisations.
- Validate all dashboard findings against the Week 6 analysis results.
- Conduct seasonal analysis broken down by appointment type.
- Define analytical testing requirements for the Week 7 brief.
- Continue collaboration with Data Science track on model evaluation results.